

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

ASTHALIN HFA INHALER

Salbutamol Pressurised Inhalation, Suspension BP 100 µg / dose

COMPOSITION

Each actuation delivers

Salbutamol Sulphate BP equivalent to Salbutamol 100 µg

Suspended in Propellant HFA-134a..... q.s

DESCRIPTION

A one piece filled metal container fitted with a valve with plastic stem and printed with product name, batch number and container number. On visual examination there should be no sign of physical damage such as dents, bulged container bent valve stem and uneven crimping. A white coloured powder is obtained after opening the container.

PHARMACOLOGY

Pharmacodynamics

Salbutamol stimulates β -adrenergic receptors and has little or no effect on the α -adrenergic receptors. Salbutamol is relatively selective β_2 -adrenergic agonist. β -adrenergic agonists stimulate the production of cyclic adenosine-3',5'-monophosphate (AMP) by activation of the enzyme adenylyl cyclase. Cyclic AMP appears to mediate numerous cellular responses. Increased intracellular cyclic AMP enhances the activity of cAMP dependent protein kinase A, which inhibits the phosphorylation of myosin and lowers intracellular calcium concentrations, resulting in smooth muscle relaxation. Increased intracellular cyclic AMP concentrations also are associated with inhibition of the release of mediators from mast cells in airways.

Pharmacokinetics

Absorption

Salbutamol appears to be absorbed from the respiratory tract over several hours following oral inhalation. The peak plasma salbutamol concentrations occur in 2.5 hours.

Metabolism and Elimination

After oral inhalation of salbutamol in healthy adults, the elimination half-life of unchanged drug was determined indirectly to be 3.8 hours. Salbutamol is extensively metabolized in the liver, mainly to salbutamol 4'-O-sulfate which has little or no β -adrenergic stimulating effect and no β -adrenergic blocking effect. Salbutamol and its metabolites are rapidly excreted in urine and faeces. After oral inhalation of salbutamol in patients with asthma, approximately 70% of a dose is excreted in urine as unchanged drug and metabolites within 24 hours and 80-100% within 72 hours; about 30% of the dose is excreted in urine unchanged in 24 hours. About 10% of an inhaled dose of salbutamol may be excreted in faeces.

INDICATIONS

Asthalin HFA Inhaler is indicated in the treatment and prophylaxis of bronchospasm in bronchial asthma, chronic bronchitis and emphysema.

DOSAGE & ADMINISTRATION

Adults and children over four years of age

For the relief of acute episodes of bronchospasm or prevention of asthma symptoms

One or two inhalations four to six times daily. If control of mild asthma deteriorates and results in regular use of a short acting β_2 -agonist (i.e 4 times daily), patients should be instructed to contact a clinician for revaluation and possible institution of maintenance therapy.

To prevent exercise-induced bronchospasm

Two inhalations 15 minutes prior to exercise.

Children below 4 years of age

Safety and effectiveness of inhaled salbutamol have not been established.

Route of Administration: Oral Inhalation

CONTRAINDICATIONS

Hypersensitivity to any of the components of the formulation.

SIDE EFFECTS

Mild tremors and headache have been rarely reported. These usually disappear with continued treatment. As with other inhalation therapy the potential for paradoxical bronchospasm should be kept in mind. If it occurs, the therapy should be discontinued immediately and alternative therapy instituted. Hypokalemia may also result from β_2 -agonist therapy. Hypersensitivity reactions including angioedema, urticaria, bronchospasm, hypotension and collapse have been reported very rarely.

DRUG INTERACTIONS

Asthalin and non-selective beta-blocking drugs such as propranolol, should not be usually prescribed together. Potentially serious hypokalemia may result from β_2 -agonist therapy. Particular caution is advised in acute, severe asthma as this effect may be potentiated by concomitant treatment with xanthine derivatives, steroids, diuretics and by hypoxia. It is recommended that serum potassium levels are monitored in such situations.

WARNINGS AND PRECAUTIONS

Asthalin preparations should be used with caution in patients suffering from thyrotoxicosis.

Nursing Mothers

As salbutamol is probably secreted in breast milk, its use by nursing mothers requires careful consideration.

Pregnancy

Administration of the drug during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

SYMPTOMS AND TREATMENT OF OVERDOSE

Overdosage of orally inhaled salbutamol produced no fatalities. The commonest symptoms were tremor, flushing, agitation and palpitations due to tachycardia. Usually no specific treatment is necessary.

In the management of overdose, the manufacturers suggest that a relatively selective β_1 -adrenergic blocking agent may be used, if necessary, but only with extreme caution in asthmatic patients because an asthmatic attack may be induced. Dialysis is not appropriate therapy for the management of salbutamol overdosage.

STORAGE CONDITION

Store below 30°C and protect from direct sunlight. Do not freeze.

SHELF LIFE

24 months

PACKAGING / PACK SIZE

ASTHALIN HFA INHALER without dose indicator are available in canisters containing 200 metered doses.

MANUFACTURER

Medispray Laboratories Pvt. Ltd.
344 / 345, Kundaim Industrial Estate,
Kundaim, GOA 403115 INDIA

PRODUCT REGISTRATION HOLDER IN MALAYSIA

CIPLA MALAYSIA SDN. BHD.
Suite 1101, Amcorp Tower,
Amcorp Trade Centre,
18, Persiaran Barat,
46050 Petaling Jaya,
Selangor.

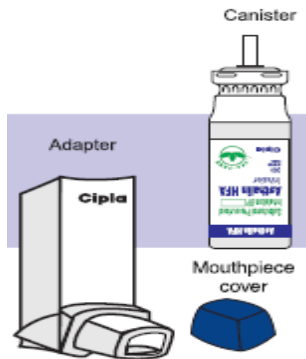
DATE OF REVISION

March 2024

ASTHALIN HFA INHALER WITHOUT DOSE INDICATOR

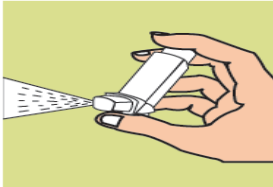
Instruction for Use

Parts of the Inhaler:



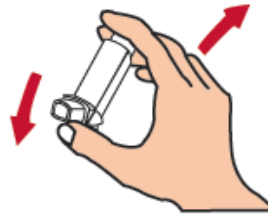
Testing your inhaler

Before using your Inhaler for the first time, or if it has not been used for a week or more, shake it well and then “test fire” it i.e., release one puff into the air.



Using your Inhaler

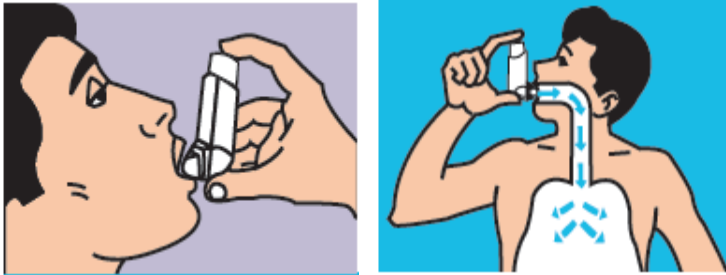
1. Remove the mouthpiece cover, and check that the mouthpiece is clean. Shake the inhaler well before each use.



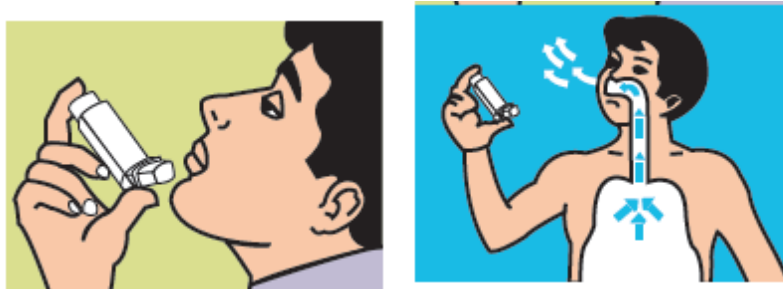
2. Hold the inhaler as shown, between index finger and thumb. Breathe out gently, through your mouth and immediately place the mouthpiece in your mouth between your teeth and close your lips around it (Do not bite it).



3. Tilt your head slightly backwards. Start breathing in slowly through your mouth. As you breathe in, press down the canister to release one dose while continuing to breathe in steadily and deeply.



4. Remove the Inhaler from your mouth and hold your breath for 10 seconds, or for as long it is comfortable. Breathe out slowly.



5. If another dose is required wait for at least one minute. Repeat steps 2 to 4. After use, replace the mouthpiece cover.



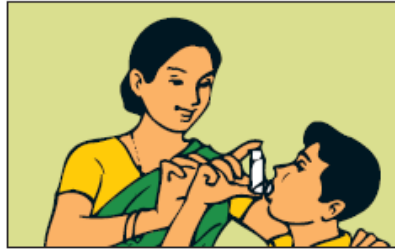
Note:

Do not rush steps 2 and 3. It is important to breathe in as slowly through your mouth just before pressing the canister. Practise in front of the mirror for the first few times.



If you see 'mist' coming from the top of the inhaler or the sides of your mouth, start again from step 1. This escaping mist indicates incorrect technique.

Parents must assist those children who need help in using the inhaler correctly.



Cleansing

Clean your inhaler at least once a week as follows:

1. Gently pull the metal canister out of the adapter of the inhaler. Remove the mouthpiece cover.
2. Rinse the adapter and mouthpiece cover in warm water. Do not put the metal canister into water.
3. Shake well to remove excess water.
4. Leave to dry in warm place. Avoid excess heat.
5. Replace the canister and the mouthpiece cover correctly.



Warning

Use only as directed by your doctor. If the recommended dose does not provide the relief of symptoms or symptoms become worse, consult your doctor.

The metal canister is pressurized. Do not puncture, break or burn it even when empty.

Keep away from eyes.

Keep away from children.